

Daily Content Intel

August 6, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, so a dose-response meta-analysis just landed in Annals of Physical and Rehabilitation Medicine, 84 studies, a little over 4,000 people, and about 1,000 of those were athletes. The question was simple. How much hamstring stretching do you actually have to do before your range of motion moves in a way anybody can measure?

Here's the number. A single quick session, roughly a minute and a half of static stretching, did not clear the minimal detectable change for active knee extension, which sat at 12 degrees. So the stretch you do on the sideline right before warmups is doing

something for how you feel, and the lasting range shows up somewhere else.

What cleared the bar was volume spread across time. Repeated stretching over at least a week, adding up to about 70 cumulative minutes, was where the gains got big enough to count. And in the athlete pool, static stretching bought about 9 degrees of active knee extension.

So if you're a football or lacrosse kid who can't get into a clean hinge, do the math with me. 70 minutes across a week is 10 minutes a day. That's 2 minutes per hamstring, twice, while you're watching film.

Small, boring, daily.

Does that make sense?

Be Good. Help Someone. Learn Lots.

Hook: 84 studies just answered how long you actually have to stretch before your hamstrings change.

Spoken script (word for word):

Okay, so a dose-response meta-analysis just landed in Annals of Physical and Rehabilitation Medicine, 84 studies, a little over 4,000 people, and about 1,000 of those were athletes. The question was simple. How much hamstring stretching do you actually have to do before your range of motion moves in a way anybody can measure?

Here's the number. A single quick session, roughly a minute and a half of static stretching, did not clear the minimal detectable change for active knee extension, which sat at 12 degrees. So the stretch you do on the sideline right before warmups is doing something for how you feel, and the lasting range shows up somewhere else.

What cleared the bar was volume spread across time. Repeated stretching over at least a week, adding up to about 70 cumulative minutes, was where the gains got

big enough to count. And in the athlete pool, static stretching bought about 9 degrees of active knee extension.

So if you're a football or lacrosse kid who can't get into a clean hinge, do the math with me. 70 minutes across a week is 10 minutes a day. That's 2 minutes per hamstring, twice, while you're watching film.

Small, boring, daily.

Does that make sense?

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On-screen text seeds:

- 0:00 84 studies. 4,000 people. One question.
- 0:08 How long do you actually have to stretch?
- 0:18 90 seconds once: below the 12 degree threshold
- 0:33 70 cumulative minutes over a week: gains that count
- 0:45 Athletes gained about 9 degrees of knee extension

- 0:58 10 min/day = 2 min per hamstring, twice
- 1:12 Small. Boring. Daily.

Caption (copy-paste ready):

New dose-response meta-analysis, 84 studies, roughly 4,000 people, about 1,000 of them athletes.

A single 90 second static stretch did not clear the minimal detectable change for active knee extension, which was 12 degrees. Repeated stretching across at least a week, totaling around 70 cumulative minutes, was where the range of motion gains got big enough to actually count. Athletes averaged about 9 degrees of active knee extension.

If your hinge is stuck, 70 minutes a week is 10 minutes a day. 2 minutes per hamstring, twice, while you watch film. That's the whole prescription.

Stretching is a dosing problem, and most athletes are under-dosed by an order of magnitude.

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Dr. Lars Stevenson, PT, DPT
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#hamstrings #footballtraining #lacrosse
#strengthandconditioning
#athletedevelopment #sportsphysicaltherapy
#rangeofmotion #hsfootball #youthsports
#speedtraining

Personalize this

- Which of your football or lacrosse athletes actually has a true hamstring range restriction versus a hinge they've never been coached through? Name the tell you use to separate the two in the first 5 minutes.
- You program speed of movement with Ian King notation. Where does 70 cumulative minutes of hamstring work per week get parked in an in-season lacrosse week without stealing from the lift?
- Which letter of CROSS does this land under for you, and do you give athletes the 10-minutes-a-day version or fold it into the warmup you already own?

Screenshots:

- title |
<https://pubmed.ncbi.nlm.nih.gov/42551249/>
 | "Optimal stretching dose for improving range of motion in non-athletes and athletes with hamstring shortening: a dose-response meta-analysis"
- physiology |
<https://pubmed.ncbi.nlm.nih.gov/42551249/>
 | "For static stretching, a brief single-session dose (1.5 min) did not exceed the minimal detectable change (12°) for AKE."
- actionable |
<https://pubmed.ncbi.nlm.nih.gov/42551249/>
 | "repeated stretching delivered over at least 1 week, with a cumulative volume of 70 min, was more likely to exceed this threshold"

Sources:

- <https://pubmed.ncbi.nlm.nih.gov/42551249/>
- <https://doi.org/10.1016/j.rehab.2026.102153>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, hot take. The 2D knee valgus screen, where somebody films an athlete doing a single-leg squat and circles the moment the knee falls in, is a rougher instrument than the confidence people have in it.

New paper in the International Journal of Sports Physical Therapy, August 1st. They ran adolescent athletes through a three-task screen, single-leg squat, single-leg drop landing, and double-leg vertical jump, with real 3D motion capture going, and then they compared the moment the knee looked most medial against the moment peak knee abduction actually happened.

Those two moments landed in different places. On the double-leg jump landing, the knee hit its most medial position at about 18 percent of the phase, and

peak knee abduction showed up at 58 percent. That's most of a landing apart.

And here's the mechanical piece. At the frame where the knee looked its worst, the tibia was rotated more externally, which runs sideways to the classic valgus story I got taught.

So keep filming it, it's cheap and it does tell you how the whole leg organizes itself. Just hold the finding loosely, because it's catching a different thing at a different moment than a knee abduction measurement, and an athlete gets cleared or held out on calls like that.

I still like the screen. I like it as a conversation starter, and stuff.

Be Good. Help Someone. Learn Lots.

Hook: The knee cave you circle on a phone video is not measuring the thing you think it's measuring.

Spoken script (word for word):

Okay, hot take. The 2D knee valgus screen, where somebody films an athlete doing a single-leg squat and circles the moment the knee falls in, is a rougher instrument than the confidence people have in it.

New paper in the International Journal of Sports Physical Therapy, August 1st. They ran adolescent athletes through a three-task screen, single-leg squat, single-leg drop landing, and double-leg vertical jump, with real 3D motion capture going, and then they compared the moment the knee looked most medial against the moment peak knee abduction actually happened.

Those two moments landed in different places. On the double-leg jump landing, the knee hit its most medial position at about 18 percent of the phase, and peak knee abduction showed up at 58 percent. That's most of a landing apart.

And here's the mechanical piece. At the frame where the knee looked its worst, the tibia was rotated more externally, which runs sideways to the classic valgus story I got

taught.

So keep filming it, it's cheap and it does tell you how the whole leg organizes itself. Just hold the finding loosely, because it's catching a different thing at a different moment than a knee abduction measurement, and an athlete gets cleared or held out on calls like that.

I still like the screen. I like it as a conversation starter, and stuff.

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On-screen text seeds:

- 0:00 That knee-cave screenshot is a rough instrument
- 0:10 IJSPT, August 1, 2026. 3D motion capture.
- 0:22 Most medial knee $\neq$ peak knee abduction
- 0:35 Jump landing: 18% of the phase vs 58%
- 0:48 At the "worst-looking" frame, the tibia was more externally rotated
- 1:02 Keep filming it. Hold it loosely.

- 1:12 Athletes get cleared on calls like this.

Caption (copy-paste ready):

New study in the International Journal of Sports Physical Therapy, August 1st. Adolescent athletes went through a three-task screen, single-leg squat, single-leg drop landing, double-leg vertical jump, with 3D motion capture running the whole time.

The moment the knee looked most medial and the moment peak knee abduction actually happened were not the same moment. On the jump landing it was 18 percent of the phase versus 58 percent. And at the frame where the knee looked worst, the tibia sat in more external rotation, which runs sideways to how dynamic valgus usually gets described.

The 2D screen is still worth doing. It's cheap, it's accessible, and it tells you something real about how the leg organizes. It's just a wider, blunter signal than a knee abduction measurement, and athletes get cleared and held out on calls like this every week.

Screen with it. Decide with more.

Be Good. Help Someone. Learn Lots.

Dr. Lars Stevenson, PT, DPT

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#acl #kneevalgus #returntosport

#sportsphysicaltherapy #lacrosse

#footballtraining #movementscreen

#athletedevelopment

#strengthandconditioning #youthsports

Personalize this

- What do you actually watch on a single-leg drop landing beyond the knee position, and which of those cues has changed a return-to-sport call you made?
- Walk through the exact battery you trust for clearing a lacrosse athlete post-ACL: strength symmetry side to side, hop and jump testing, change of direction under fatigue, movement quality, psychological readiness. Which one has caught someone the eyeball screen missed?
- Do you say anything to the athlete when their knee cave looks ugly on video, or do you keep that between you and the plan?

Fear of the picture is its own problem.

Screenshots:

- title |
<https://pubmed.ncbi.nlm.nih.gov/42549198/>
| "Does Medial Knee Position Reflect Dynamic Knee Valgus? Comparing Kinematic Timing and Magnitude During a Three-Task Movement Screen"
- physiology |
<https://pubmed.ncbi.nlm.nih.gov/42549198/>
| "Across all tasks, tibial rotation was more external when the knee was at its most medial position."
- actionable |
<https://pubmed.ncbi.nlm.nih.gov/42549198/>
| "2D peak knee medialization provides a clinically accessible tool to assess broader movement patterns of the lower extremity whenever 3D analysis is not feasible"

Sources:

- <https://pubmed.ncbi.nlm.nih.gov/42549198/>
- <https://ijspt.scholasticahq.com/>

Reference

Runner-up topics

- Non-functional overreaching in NCAA D1 football across a 15-week season (PMID 42553027): non-starters showed physiological strain patterns despite lower external load, and mid-season CMJ declines tracked with biomarker shifts. Strong "the backups are getting cooked too" angle for HS football staffs.
- Trajectories of 10 m sprint time and CMJ height relative to peak height velocity in male youth soccer players (PMID 42555795): the growth-spurt performance dip, and what to program through it.
- Nutrient intake and nutrition knowledge by sport type among adolescent athletes (PMID 42552913): fueling gaps by sport, straight into the under-fueling lane.

Sources scanned today

- Newsletter digest
newsletter_digest_2026-08-06.pdf: 1 source (Dr. Mercola promo blast), 0 educational links. Pure advertorial

(berberine, NAC, probiotics). Nothing usable, skipped.

- PubMed, recency-sorted sweep on athlete / strength and conditioning / sprint / return-to-sport: 25 most recent, nearly all within 10 days. Best hits: hamstring stretching dose-response meta-analysis (Aug 4), medial knee position vs dynamic knee valgus screen (Aug 1), non-functional overreaching in D1 football (Jul 21), 10 m sprint and CMJ trajectories around peak height velocity in youth soccer (Aug 5).
- PubMed, second sweep on youth / adolescent / high school / football / lacrosse + training / injury prevention / performance: surfaced the D1 football overreaching paper, adolescent athlete nutrition knowledge study, ACL injury by playing position in football, proximal femoral growth plate FEA in adolescent athletes.
- BJSM targeted search: no distinct August 2026 return-to-sport item surfaced above what PubMed already gave. Skipped.

- ScienceDirect (Annals of Physical and Rehabilitation Medicine full text): HTTP 403, publisher paywall. Abstract read on PubMed instead.
- JOSPT (Rethinking Dynamic Knee Valgus, 2019): HTTP 403, blocked. Held as background only, not cited.
- Europe PMC mirror: returned navigation shell, no article content. Skipped.
- IJSPT Scholastica search: search interface returned no parseable results. PubMed record used instead.